



## POLICY AND PROCEDURE

# Management of Restrictive Practices

(Seclusion and Restraint)

|                       |                                                   |
|-----------------------|---------------------------------------------------|
| <b>Scope (Staff):</b> | All NMHS Mental Health Workers                    |
| <b>Scope (Area):</b>  | All NMHS Mental Health Authorised Inpatient Areas |

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## 1. Aim

North Metropolitan Health Service (NMHS) Mental Health (MH) Inpatient Services aim to reduce and where possible, eliminate the use of restraint and seclusion practices along with other forms of restrictive practices. MH is committed to providing least restrictive care that is trauma informed and recovery focused.

The policy and procedure provide direction for workers in the use of physical restraint and seclusion in authorised MH Services as well as providing guidance to ensure other restrictive practices do not occur within Inpatient areas. For non-authorised settings please refer to the [Chief Psychiatrist's Guideline issued under Section 547\(3\) and 548 of the Mental Health Act 2014](#). This guideline sets out the legislative requirements for the lawful use of detention and restraint in a non-authorised healthcare setting, within a human rights and best practice based framework.

Whenever practicable, clinicians will avoid and eliminate the use of physical restraint and seclusion. Early assessment and intervention, collaboration, crisis management, de-escalation and other supportive alternatives are preferred interventions of choice in delivering safe, high quality patient care, whilst also maintaining a safe workplace.

## 2. Background

Where necessary to protect the safety of patients and workers the use of restrictive interventions, such as restraint and seclusion, must, if required, be administered safely and for the shortest practicable period. Seclusion and restraint are to be carried out by appropriately trained workers in accordance with the requirements of the [Mental Health Act 2014](#) (MHA 2014) and the [WA Health Code of Conduct Policy](#).

According to the Western Australia (WA) [Chief Psychiatrist's Standards for Clinical Care](#) (s.547 [MHA 2014](#)) seclusion and restraint are interventions and not therapies and their use in mental health facilities increase the risk of trauma and physical harm to both workers and patients. Services should therefore aim to reduce and where practicable eliminate both restraint and/or seclusion, time spent in seclusion and trauma associated with restraint, seclusion, and other restrictive practices. Where there are no appropriate alternatives to seclusion or restraint, they should be administered in the safest, most dignified, compassionate, and respectful manner as possible by appropriately trained workers. Services should also ensure processes within their areas of responsibility to examine other areas of practice that may be considered restrictive in nature.

Clinical care will focus on the elimination of restrictive practices in the treatment setting. It will be presumed that every patient has been exposed to trauma in the past and as such all restrictive practices will be in accordance with trauma informed principles. Please refer to the [Office of the Chief Psychiatrist of Western Australia Restrictive Practices Factsheet](#).



### 3. Risk

Non-compliance with this policy will impact on the following risk categories:

|                                       |                                     |                                 |                                     |
|---------------------------------------|-------------------------------------|---------------------------------|-------------------------------------|
| Culture                               | <input checked="" type="checkbox"/> | Organisational Development      | <input type="checkbox"/>            |
| Emergency Management                  | <input checked="" type="checkbox"/> | Policy & Legislative Compliance | <input checked="" type="checkbox"/> |
| Fiscal Responsibility                 | <input type="checkbox"/>            | Quality of Patient Care         | <input checked="" type="checkbox"/> |
| Governance & Accountability           | <input checked="" type="checkbox"/> | Research & Innovation           | <input checked="" type="checkbox"/> |
| Information Communications Technology | <input type="checkbox"/>            | Staff Safety & Wellness         | <input checked="" type="checkbox"/> |
| Facility Management                   | <input type="checkbox"/>            | Workforce Planning              | <input type="checkbox"/>            |
| Medical Equipment                     | <input type="checkbox"/>            | Other                           | <input checked="" type="checkbox"/> |

### 4. Definitions

**Authorisation of Restraint:** Restraint can only be used following authorisation by a Medical Officer (MO) or Mental Health Practitioner (s.230, s.231 *MHA 2014*).

**Bodily Restraint:** is the physical or mechanical restraint of a person who is being provided with treatment or care at an authorised hospital.

Bodily restraint does not include:

- physical or mechanical restraint by a police officer acting in the course of duty; or
- physical restraint by a person exercising a power under section s.172(2) *MHA 2014*.

**Breaking of Seclusion:** Interrupting or terminating seclusion.

**Physical Restraint:** is defined as bodily force to the person's body to restrict a person's movement (s.227(2) *MHA 2014*).

The exception to s.227(2) *MHA 2014*, is if the person is provided with physical support or assistance to:

- redirect the person because the person is disoriented; or
- to enable the person to carry out activities of daily living.

These types of physical support or assistance are not bodily restraint (s.227(3) *MHA 2014*).

**Mechanical Restraint:** is defined as the application of a device to restrict a person's movement, such as a belt, harness, sheet or strap. Medical restraint does not include either the appropriate use of a medical or surgical appliances or the appropriate use of furniture to restrict a person such as cot sides or a chair fitted with a table. It also does not include physical or mechanical restraint by a police officer. (s.224 (4) *MHA 2014*).

**Medical Practitioner:** A person registered under the Health Practitioner Regulation National Law (Western Australia) in the medical profession and registered with the Australian Health Practitioner Regulation Agency (AHPRA).

**Mental Health Practitioner:** A person who is qualified as one of the following and has at least 3 years' experience in the management of people who have a mental illness:

- 
- Nurse whose name is entered on Division 1 of the National Register of Nurses under the Health Practitioner Regulation National Law (WA) as a registered nurse with the Australian Health Practitioner Regulation Agency (AHPRA)
  - Clinical Psychologist (Registered with AHPRA)
  - Occupational Therapist (Registered with AHPRA)
  - Social Worker

**Nominated Person:** Means a person nominated under s.263(1) of the *MHA 2014*. A nominated person may be the primary carer or any other person who is formally nominated by a person experiencing mental illness. A nominated person is entitled to be informed and involved whilst the patient is being provided with treatment or care.

**PART:** Predict Assess Respond To (PART) Challenging and Aggressive Behaviour Training is approved as the mandatory training program that is provided for all workers working in MH services.

**Personal Support Person:** means a close family member, carer, nominated person, parent or guardian of a child and guardian or enduring guardian of an adult who provides or has provided in the past, ongoing personal care, support, advocacy and/or assistance to a person with mental illness.

**Restrictive practices:** A restrictive practice is any action that restricts the rights or freedom of movement of a care. There are five types of restrictive practices: chemical, environmental, mechanical, physical restraint and seclusion.

**Restriction on Freedom of Communication:** As per s.262 of the *MHA 2014* a patient may have their freedom of communication restricted by their Consultant Psychiatrist if it is deemed in the best interests of the patient. For the purpose of this document the forms of communication which may be restricted include:-

- seeing and speaking with other people in the hospital to the extent that this is reasonable;
- receiving and sending of postal mail and/or electronic communications;
- receiving visitors and having contact from other persons;
- making and receiving telephone calls.

**Seclusion:** s.212 of the *MHA 2014* defines seclusion as the confinement of a person who is being provided with treatment or care at an authorised hospital, by leaving the person at any time of the day or night alone in a room or area from which it is not within the person's control to leave. A person is not secluded merely because the person is alone in a room or area that the person is unable to leave because of frailty, illness or mental or physical disability.

**Time out:** 'Time out' is not defined in the *MHA 2014*. It may be part of a treatment plan or a behaviour modification strategy as a way of responding to heightened emotions expressed by angry feelings or signs of distress, whereby the patient agrees to go into a pre-determined 'time out' room that is safe and where the patient can exit, is in agreement with the plan and should not be prevented from freely leaving the room at any time.



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In a non-authorised facility, placing a person alone in a room from which they cannot leave may be a breach of s.333 of [The Criminal Code](#) and is never “time-out”. A person may only be placed in a room from which they cannot leave if there are justifiable and proper reasons for doing so. These may include exercising a duty of care to the patient or others and where there is imminent danger with no safe alternative. This is an intervention in an emergency and any such intervention needs to be accurately documented in the patient’s clinical record with the treating Psychiatrists or Duty Consultant notified and the patient reviewed as soon as practicable.

**Restraint:** Bodily restraint is the physical or mechanical restraint of a person who is being provided with treatment or care at an authorised hospital.

A person is not being physically restrained when being provided with the physical support or assistance reasonably necessary to enable the person to carry out daily living activities or to redirect the person because the patient is disoriented (s.227 MHA 2014). For the purposes of this policy bodily restraint does not include:

- Physical or mechanical restraint by a NMHS Security Officer(s) or Police Officer acting in the course of duty;
- Physical restraint by a person exercising a power under s.172(2) MHA 2014.

**Worker:** any person performing duties on behalf of NMHS. This may include direct employees, contract or agency workers, volunteers, or students. (Note: NMHS has a duty of care under the [Work Health and Safety Act 2020](#) and the [Work Health and Safety Regulations 2022](#) to provide a safe workplace as far as is reasonably practicable to workers and other persons: this includes visitor and other patients.)

## 5. Principles

NMHS MH supports the principle that restraint and seclusion must not be listed as a planned intervention. MH is also committed to providing clinical interventions that are therapeutic, patient centred, evidence-based, trauma informed, recovery focused and in collaboration with patients, family, and carers.

NMHS recognises that collaboration with the patient, family and carer during the assessment, care planning and ongoing treatment process improves the experience for those involved and delivers outcomes that are more positive for patients, family, and carers.

In accordance with the [Chief Psychiatrist’s Guideline for the Sexual Safety](#) of consumers of Mental Health Services, where seclusion or restraint is used, sensitivity must be applied to reduce the risk of retraumatising and it must be undertaken in a manner that respects the dignity of the patient (e.g. patient is adequately clothed; clinician of the same gender present). It must be carried out by appropriately trained staff consisting of clinicians and may include security staff.

If there are concerns that a situation is escalating, or if consideration is being given to the use of restraint and/or seclusion, the Shift Co-ordinator must notify the Nurse Manager (NM), Clinical Nurse Specialist (CNS) or Clinical Nurse Manager (CNM) after hours who will provide advice and support and attend the clinical area as a matter of urgency. The purpose of this clinician attending is to assess and ensure all de-escalation and/or alternative interventions have been attempted. Consideration should also be given to early escalation for medical review.





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Clinical workers should collaborate with the patient, family and carer during the assessment, care planning and ongoing treatment process improves the experience for those involved and delivers outcomes that are more positive for patients, family, and carers.

Clinical workers will ensure a risk assessment is undertaken. Assessment is undertaken utilising standardised risk assessment process and tools identified in the [State-wide Standardised Clinical Documentation](#) (SSCD) and the [MH Clinical Risk Assessment and Management Policy](#) (CRAM). Assessments will be undertaken within a specified timeframe by appropriate members of the multidisciplinary team.

A PSOLIS (Psychiatric Services Online Information System) Treatment, support and discharge Plan and Coping and Safety Tool for Awareness and Recovery (**PMR 10 Appendix 3**) will be completed with the patient as soon as practical on admission, identifying, wherever possible, the precipitating and exacerbating factors relevant to patient safety and outlining a graded series of possible responses. Consumer Safety Plans will be reviewed regularly and after an incident of either seclusion and/or restraint with the patient.

Workers training will comply with mental health legislation requirements relating to the use of seclusion and restraint and will be standardised to minimise errors according to the [WA Chief Psychiatrist's Standards for Clinical Care](#) (s. 547 *MHA 2014*), **focussing heavily on de-escalation.**

NMHS MH also recognises their duty to provide a safe workplace to all workers and other persons. The proportionality of risk to patient must be considered against the risk to workers when undertaking any activity in the workplace, such as interacting with a person exhibiting behaviours that may place themselves or others at risk.

## 5.1 Restraint Principles

The use of restraint in Authorised Mental Health Services is tightly regulated by the *MHA 2014*, Division 6 - Bodily Restraint and is monitored and reviewed by the Chief Psychiatrist of WA.

The use of handcuffs as a mechanical restraint for Mental Health Inpatients is **not** to be authorised by Nursing staff. The use of handcuffs for persons other than Inpatients under the care of the Frankland Centre will be executed by competent Security Officers and reported under the [NMHS Use of Force Policy](#)

If an intervention involves using bodily force to restrain a person, then it meets the definition for “*restraint*” as defined in the *MHA 2014* and must be reported to the Chief Psychiatrist.

All workers must have knowledge of the Legislative and Policy requirements regarding the use of restraint and its application, and adhere to the mandatory reporting requirements for all incidents of patient restraint in Authorised Mental Health services.

Restraint will only be used when all less restrictive interventions have been unsuccessful. If restraint is required it should only be used because:

- There is an imminent risk of danger to the individual or others and no other safer intervention is identified at the time.
- There is a need for the administration of a prescribed treatment following the unsuccessful exploration and attempts of all other least restrictive alternative interventions.

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If used, the restraint interventions must be administered safely, for the least practicable amount of time and with the least amount of physical force required, by appropriately trained workers and in accordance with the WA Health Code of Conduct.

Workers must be aware of prior and present patient risk factors which may result in complications during and following the use of physical restraint (e.g. respiratory problems, cardiac conditions, substance withdrawal, pregnancy).

Restraint should never be used for the purpose of discipline, coercion or as a way of addressing staffing issues such as inadequate staffing.

Where a risk of restraint occurring is identified, workers will as soon as practicable ascertain fitness to be restrained, including cardiac status by referring to the Physical Examination Form (**PMR 5A**). Alternative strategies to restraint may include a period to allow for de-escalation interventions including use of medications, crisis management interventions or other alternatives identified by the patient.

In situations where it is considered unsafe to implement a restraint intervention evacuation may be considered as the safest option. This action allows time for the Coordinating Clinician to consult with the Medical officer (MO) and senior nurse to assess if there is any imminent danger and explore other options such as, waiting for more workers to become available, calling Security, contacting the Police or seeking other outside assistance. In the event Security Personnel attend a potential restraint event, the role of Security workers is to assist the clinical team under the direction of the Coordinating Clinician, in accordance with local service policies. Security staff will receive an appropriate handover and will understand their allocated role.

## 5.2 Seclusion Principles

Seclusion is a safety procedure of last resort, not a treatment intervention. Seclusion should never be used for the purpose of discipline, coercion or as a way of addressing staffing issues such as inadequate staffing. The use of seclusion and restraint creates significant risks for people whose behaviour is creating a serious and immediate risk for themselves or others. These risks include serious injury or death, re-traumatisation for people who have a history of trauma, loss of dignity and other psychological harm. Considering these consequences, seclusion should only be used only when there exists an imminent risk of danger to the individual or others and no other safe and effective intervention is possible. Seclusion should be used for the least time possible and terminated when the criteria for using seclusion is no longer satisfied. Every 15 minute check should be utilised as an opportunity to review and cease seclusion.

Where a patient is in seclusion for greater than 24 hours care must be escalated to the Mental Health Advocacy Service (MHAS) and the Chief Psychiatrist at regular intervals.

## 5.3 Restriction on Freedom of Communication – Form 12C

As per s.262 of the *MHA 2014* a patient may have their freedom of communication restricted by their treating Psychiatrist if it is deemed in the best interest of the patient.

The decision to restrict a patient's communications may only be made by the treating/duty Psychiatrist in consultation with the treating team and the patient. The Mental Health Tribunal may be asked to review the matter and can overturn the decision.



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There are no absolute contraindications for the restriction of a patient's freedom of communication. The ability to restrict a patient's communication **excludes** contact with the Mental Health Advocacy Service (MHAS) or Legal Practitioner unless the treating Psychiatrist deems there is a risk of harm.

Any order made to prohibit or limit a patient's rights of freedom of lawful communication must be reviewed at least every 24 hours, otherwise it ceases to be in force.

Next of kin, carers, significant others will be informed of the restriction on communication by the shift coordinator, with consideration of consent issues.

Please refer to the [MH Patients' Freedom of Lawful Communication Policy](#)

## 6. Strategies to Improve Outcomes for Patients

Best practice includes the involvement of the patient and carer in their treatment and care. The National Safety and Quality Health Service ([NSQHS Standards](#)) provide a nationally consistent statement of the level of care patients can expect from health service organisations. This is supported by the [User guide for acute and community mental health services](#). The responsible clinician will ensure the patient and their carer (with patient consent) is involved in making choices in their treatment options and the planning of their care where possible. Multidisciplinary Clinical Reviews must be undertaken regularly and the PSOLIS Treatment, Support and discharge Plan (TSDP) and a Coping and Safety Tool for Awareness and Recovery (**PMR 10**) are developed progressively and regularly updated with the involvement of the patient and with the permission of the patient includes their family, carer or nominated person. If it is not possible to involve the patient, family, carer or nominated person the reason will be documented in the patient's medical record.

Early identification and risk assessment of individuals who may require restraint and/or seclusion as a last resort is essential and all attempts should be made to avoid both events. Restraint and/or seclusion should not be identified as a planned intervention.

### 6.1 Prevention, Early Intervention and De-Escalation Strategies

The role of the mental health worker is to assist the consumer to learn how trauma related behaviours (e.g. aggression) have developed and to teach them new positive coping strategies (e.g. self-modulation, self-regulation). This requires frequent positive feedback and open communication to encourage/motivate the consumer to reshape their behaviours. Discussion regarding positive behaviours and activities that will promote self soothing and management of distress, anger and/or frustration should occur at multiple points in the patient's journey and be related to their individual experiences. Supportive observation and engagement, is also a therapeutic intervention aimed at reducing factors which contribute to increased risk. What keeps people safe is not the act of being under surveillance (observation); rather it is the quality of engagement between that individual and others.

The following strategies are encouraged:

- Evidenced based, individualised, appropriate treatment interventions conducted by skilled and competent workers who recognise escalation and utilise alternative strategies to prevent and defuse escalating situations.
- Collaborative care planning and identification of effective alternatives to manage distress and agitation when it arises.





- Use of resources from sensory trollies/boxes or access to quiet or sensory areas where available.
- Physical presence of clinical staff including the Peer workforce where appropriate within the ward environment.
- Support patients to achieve their optimal level of activities of daily living.
- Integrated and comprehensive strategies including all members of the care team and including carers and peer support wherever possible
- Respecting the patient's right to receive respectful and culturally secure care in the least restrictive environment.
- Adherence to the *MHA 2014*, policies and procedures that clearly identify that restraint and seclusion must only be used as a last resort and in response to safety concerns that cannot be managed in a therapeutic and less restrictive manner.
- Use of continuous evaluation, audit and review mechanisms to ensure standards are met and to provide evidence of continuous quality improvement including up to date information provided at federal and state levels on the management practices and documentation of restraint.
- Each site is required to ensure a process is embedded that includes reporting on restraint and seclusion events and provides an avenue for escalation and the review of other restrictive practices being used, for example unnecessary ward or environmental rules.
- Patients/carers/Peer Support Provider's will be made aware of the Health Service's patient feedback/complaints process should they have any concerns to be addressed.

## 7. Worker Roles

3 day Predict Assess Respond To (PART) Challenging and Aggressive Behaviour Training is the mandatory training provided for workers working in MH services who may be involved in the use of restraint and/or seclusion and outlines the roles as described below. Training will comply with mental health legislation requirements relating to the use of seclusion and restraint and will be standardised to minimise errors according to the WA [Chief Psychiatrist's Standards for Clinical Care](#) (s.547 of the *MHA 2014*) and equally contain relevant content regarding the importance of prevention of aggression, early intervention and the use of de-escalation in a crisis situation.

### 7.1 Coordinating Clinician

A Mental Health Clinician who has completed 3 day PART will coordinate the emergency, alert the Medical Officer (MO) and delegates specific roles such team leader, scribe, and management of the environment. The Coordinating Clinician is responsible for communicating with the team for the duration of the restraint. Consideration should always be given as to whether it is safe to implement techniques relating to restraint and/or seclusion, including assessing the number of workers available and their individual competencies.

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## 7.2 Team Leader

The Team Leader is nominated by the Coordinating Clinician and is situated at the head of the patient in a three person team (or on the arm in a 2 person team) and is responsible for monitoring the patient's safety during the restraint process.

The Team Leader's primary role is to:

- Monitor the patient's physiological status (such as monitoring respiration).
- Minimise the impact of the restraint.
- Maximise the space available to the patient.
- Communicate and support the patient and establish rapport.
- Direct the team to ensure safe physical restraint techniques are applied.
- Communicate with team members.
- Consider the use of alternatives to restraint, and/or terminate the restraint at any point if they assess it is safe to do so.

## 7.3 Scribe

The scribe is allocated by the Coordinating Clinician and is responsible for:

- Documenting the events as they occur in real time on the Physical Restraint form (**Form 10D**).
- Ensuring the recording of times are consistent across all documentation.
- Advising the Team Leader of any concerns about the patient's safety and/or any imminent or ongoing threat to the safety of workers/others
- Alerting the Team Leader of each minute that has elapsed and the length of time that has elapsed since the commencement of the restraint and until the cessation of the restraint.
- Considering alternatives to where prone restraint is being used and advising of time lapsed if the prone restraint position has reached three (3) minutes.
- Ensuring during physical restraint, under no circumstances is direct pressure applied to the neck, trachea, thorax, abdomen, back or pelvic areas.

## 7.4 Others

Other workers available may undertake the following duties under the direction of the Coordinating Clinician:

- raising the emergency response if required;
- providing ongoing care for other patients in the vicinity;
- removing bystanders to maintain privacy and dignity of the patient and safety for all involved in the restraint;
- offering support to others as required;

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- notifying the NM, CNS, Medical team and identified family/carers and nominated persons of the incident;
  - checking, preparing and administering prescribed medications (within scope of practice); and/or
  - other tasks as directed by the emergency response Coordinating Clinician.

## 8. Restraint and Seclusion Processes

Clinical workers will ensure a current up to date risk assessment is undertaken to ascertain the potential for aggression or violence. Assessment is undertaken utilising the Risk Assessment and Management Plan (RAMP) documentation. Assessments will be undertaken within a specified timeframe and by the appropriate members of the multidisciplinary team.

Medical Practitioners or Mental Health Practitioners must consider all least restrictive interventions prior to restraint and/or seclusion being considered. Prevention and early intervention strategies include but are not limited to use of the Coping and Safety Tool, the TSDP, diversional activities, sensory modulation, medication, or use of a low stimulus environment. Use of the Peer Support Worker or an [Aboriginal Health Liaison Officer](#) should also be considered where appropriate to attempt to engage with the patient. Implementation of the patient's and/or carer's previous advice regarding the appropriate strategy to reduce distress and agitation should also be used where possible, or their contact if appropriate.

If patients are agitated or emotionally dysregulated, workers will consider using de-escalation techniques and strategies available where they are more likely to regain control. Medical review and review of medications and discussion of pharmacological options should also be requested.

A Medical Practitioner or Mental Health Practitioner (in an emergency) is not to give authorisation to restrain and/or seclusion involving a patient unless it is necessary for the protection or safety of:

- the patient; or
- another person with whom the patient is likely to come into contact with if not restrained and/or secluded; or
- there is a need for the administration of a prescribed treatment (restraint only).

### 8.1 Restraint Authorisation Process

Restraint may only be used when other strategies have been unsuccessful and there exists an imminent risk of danger to the patient or others and no other safe and effective intervention is practicable.

If the person needs to be restrained urgently, a Medical Practitioner, Mental Health Practitioner or the person in charge of the ward can make an oral authorisation (**Form 10A**) or written authorisation (**Form 10B**) for the person to be placed into restraint. The authorisation should state whether physical or mechanical restraint can be used and if mechanical restraint, then the device which can be used and the way in which it can be applied, noting the use of mechanical restraint is only approved for Forensic Inpatients.

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In all instances where restraint is initiated in an emergency by a mental health practitioner or an appropriately trained mental health clinician, a Medical Practitioner will be informed as soon as practicable and within at least 30 minutes. The Medical Practitioner will attend, assess the patient face to face, including assessment of cardiac status, and document ongoing management of the patient in the patient's medical record which may include an instruction to vary or revoke the authorisation for restraint.

In all instances where restraint is initiated in an emergency, the most senior registered nurse on the ward/unit/site will be informed at the earliest practicable opportunity and is required to attend the restraint as soon as practicable.

As soon as practicable after an oral restraint order is authorised, the worker who made the authorisation must record the use of restraint on the Restraint Form (**Form 10A**).

Restraint must only be authorised for the shortest period and no more than 30 minutes in order to maintain the safety of the patient or another person with whom the patient might come into contact. Incidents of seated restraint that may potentially extend beyond 30 minutes should be escalated for Duty Medical Officer (DMO) and senior nursing review.

The patient must be examined by a Medical Practitioner (not necessarily the practitioner who authorised the restraint) within 2 hours and this is recorded on the Restraint Form (**Form 10E**).

Mental health practitioners or persons in charge of the ward should only use oral authorisation if the restraint is urgent and there is no medical practitioner available to give oral authorisation. The reasons for the urgency must be recorded on the Restraint Form.

As soon as practicable after the restraint has been applied (or the patient has been released if the restraint is for a short time), the person who authorised the restraint must complete a restraint form (**Form 10B**).

The person who authorised the restraint (if not a medical practitioner) must, as soon as practicable, record the medical practitioner's name and qualifications and the date and time they were informed on the restraint form (**Form 10C**).

The person who authorised the restraint (if not the Treating Psychiatrist or the medical practitioner informed as above) must inform the Treating/ Duty Psychiatrist within 30 minutes of the restraint and record the treating psychiatrist's name and qualifications and the date and time they were informed on the restraint form (**Form 10C**).

In accordance with the patient's consent and care plan, the family/support person/carer or nominated person is to be informed as soon as practical of use of restraint and when the use of restraint has been terminated.

## 8.2 Restraint to Administer Prescribed Medication

If a restraint is used because a patient will not comply with treatment and needs to be restrained for treatment to be given, a medical practitioner, mental health practitioner or person in charge of a ward will complete a written authorisation for restraint (**Form 10B**).

In the event a restraint is ordered by the Medical Practitioner to enable the administration of a prescribed treatment which is deemed necessary for a patient's recovery, the Medical Practitioner will:

- discuss and explain the need for the treatment with the patient;



- discuss and explain the need for treatment with the carer and/or guardian;
- discuss and explain to the patient, carer and guardian why a restraint is required and, when clinically appropriate, engage Peer Support Workers. Engagement with the Peer Support Worker will be dependent on the consent of the patient and the discretion and availability of the Peer Support Worker;
- ensure all least restrictive alternatives are attempted and documented in the patient's medical record as stated above;
- authorise the restraint; and
- be present during the restraint.

The Medical Practitioner will not authorise a restraint until there is documented evidence of the above steps being undertaken.

Following the restraint, the Medical Practitioner or relevant person will undertake and complete the post restraint interview with the patient. In accordance with the patient's wishes and care plan, the family/support person/carers or nominated person is to be included in the post restraint interview. A refusal to complete this should be documented in the patient's file

### 8.3 Confirmation, Extension, Variation, Revocation or Expiry of Bodily Restraint

Within 30 minutes of authorisation, a Medical Practitioner will examine the patient and confirm if the restraint should continue or be ceased.

If the restraint is to be revoked, the Medical Practitioner will complete the required Form **(Form 10G)** and ensure an examination is carried out **(Form 10I)**.

If the restraint is to be extended, the Medical Practitioner will complete the required form **(Form 10E)**, stating their view that the patient should continue to be restrained and thereby making an order extending the bodily restraint order. Variations to the bodily restraint order are to be recorded **(Form 10F)**. Restraint can only be ordered for periods of 30 minutes and the patient must be examined by a Medical Practitioner before any extension. The Medical Practitioner must examine the patient immediately before any extension of the restraint order (s234). A Medical Practitioner must examine the patient at least every 30 minutes during a series of restraint orders.

If the restraint continues for more than 6 hours, a psychiatrist must review the authorisation for restraint and make a record of that review **(Form 10H)**.

Following all restraints (including if restraint has already ceased prior to review by the Medical Practitioner), the Medical Practitioner will, within 6 hours of release, conduct an examination to ensure the patient has not suffered any complication or deterioration of their mental or physical state **(Form 10I)**.

### 8.4 Management of the Patient During Restraint

- A Mental Health Practitioner, must always remain with the patient while the patient is being restrained, ensuring the safety of the patient and monitoring the patient's responses.



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- The Team Leader is responsible for ensuring the patient's wellbeing and monitoring of their physiological status to ensure continual assessment and that their breathing is not compromised.
  - Observations should include ensuring a clear airway and that the restraint is not causing pain or that any limbs are being impacted in a negative way. Observations should be recorded on the relevant form (**Form 10D**).
  - All workers involved in restraint will have functional anatomy and physiology knowledge and understanding, including the potential physical risks of restraint, that is, asphyxiation, reduction in lung capacity particularly during prone restraint, catecholamine rush, metabolic acidosis, cardiac arrhythmias, and rhabdomyolysis.
  - A Scribe will be nominated by the Coordinating Clinician and is responsible for recording in real time details of the restraint on the Physical Restraint form (**Form 10D**).
  - The Clinical Nurse Specialist (CNS)/Nurse in Charge of the Ward (after hours the most Senior Registered Nurse on duty) is to be informed of all restraint events at the earliest opportunity and will personally attend the ward as soon as practicable to assess the situation and provide support and advice as required.
  - The relevant CNS/ Nurse in Charge of the Ward (after hours the most Senior Registered Nurse on duty) will sign the Physical Restraint form (**Form 10D**) as verification of the completion of documentation.
  - The minimal amount of force necessary, for the shortest practicable period of time required to ensure the safety of the patient or another person, will be used at all times.
  - The patient will be continually assessed to monitor for signs of physical deterioration. The restraint will be ceased immediately if the patient becomes physically compromised (e.g. vomiting, any indication of breathing difficulties, any changes to level of consciousness).
  - During restraint, the patient must be provided with the care that is appropriate to their immediate needs particularly if the restraint is required for a continued period:
    - appropriate clothing;
    - sufficient food and drink;
    - access to toilet facilities; and
    - any other issues of concern for the patient.

## 8.5 Position of the Patient During Restraint

- The restraint should be managed in a standing, sitting, kneeling or lying position. Prone restraint must be avoided wherever possible
- If prone restraint is required to safely contain the situation, it should **not exceed 3 minutes** because of an increased risk of hypoxia to the patient.
- If the use of prone restraint exceeds 3 minutes further prone restraint must be authorised by the most senior clinician in attendance.

- The Coordinating Clinician, Team Leader and MO will be required to take an active and urgent role in identifying strategies to cease prone restraint at the earliest opportunity, to assess the patient's physical health status, monitor compliance with ceasing use of the prone position after 3 minutes and justification and authorisation of any extended use of the prone position.
- Crossing the legs and positioning them near the buttocks **must not** be used during the restraint unless the low-risk strategy (straight legs) is deemed too high a risk and is required to safely exit a seclusion room. It must be utilised for **no longer than 30 seconds**.
- Following restraint, all patients must be offered post restraint support and counselling (see section 9).
- In accordance with the patient's request and care plan, a carer, close family member or nominated person Personal Support Person (PSP) should be informed 'as soon as practical' of the use of restraint and when the restraint has been terminated.
- All contacts with family, carer, nominated person must be documented in the patient's clinical record, identifying the carer and the time of the contact. If the patient does not wish to have the PSP informed, this is to be documented in the patient's clinical records.

The following additional safety principles are to be observed for all restraint positions:

- Never occlude/cover the nose and mouth unless medically approved e.g. oxygen mask.
- During physical restraint under no circumstances should direct pressure be applied to the neck, trachea, thorax, abdomen, back or pelvic areas.
- The patient's airway and head must always be protected and monitored to reduce the risk of asphyxiation or head trauma.
- Do not use restraint in a way that interferes with the patient's ability to communicate.
- Neck holds will not be used.
- Never pull on, rotate or flex the neck in any direction.
- Never apply head locks, choke holds or similar.
- Avoid hyperflexion of the trunk in standing or seated restraints.
- Continually assess the airway and protect the head if necessary.
- Consider the possibility of pregnancy and/or history of sexual assault/trauma. In these situations the use of prone or supine restraint should be avoided.
- Communication with the patient should be centralised through the Team leader where possible.

## 8.6 Authorisation of Seclusion

Seclusion may only be used when all other strategies have been unsuccessful and there exists an imminent risk of danger to the individual or others and no other safe and effective intervention is practicable. Seclusion should not be used for the sole purpose of preventing imminent self-harming behaviours. Seclusion is not an effective intervention to prevent self-

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harming behaviours as patients are able to engage in self-injury whilst in seclusion. Other strategies to manage self-harming behaviours should be utilised.

If the person needs to be secluded urgently, either a medical practitioner, mental health practitioner or the person in charge of the ward can make an oral authorisation (**Form 11A**) or written authorisation (**Form 11B**) for the person to be placed into seclusion.

As soon as practicable after an oral seclusion order is authorised, the worker who made the authorisation must record the use of seclusion on the Seclusion Form (**Form 11A**). This form gives details of the seclusion explaining why it was necessary, outlining the urgency which required an oral authorisation and detailing where the person was secluded.

If a person is restrained, and then a decision is made to also place the person in seclusion, then both a decision to restrain and a decision to seclude have been made and both forms need to be completed.

Seclusion must only be authorised for the shortest period required to preserve the safety of the patient or another person with whom the patient might come into contact.

The patient must be examined by a medical practitioner (not necessarily the practitioner who authorised the seclusion) within 2 hours.

Seclusion can be extended (**Form 11E**) for periods of up to 2 hours. An examination by a medical practitioner is always required before seclusion is extended.

Revoking or expiry of a seclusion order must be recorded on Form 11F.

The person who authorised the seclusion must inform the patient's psychiatrist/duty psychiatrist of the seclusion within 2 hours (**Form 11C**).

The relevant Clinical Nurse Specialist (CNS) (after hours the most senior registered nurse on duty) is to be informed of all seclusion events. The CNS (after hours the most senior registered nurse on duty) will attend, sight the patient and confirm the correctness of the nursing actions and documentation and support as required.

## 8.7 Management of Patients in Seclusion

All hazardous items (such as belts, shoelaces, dressing gown or pyjama strings, pens or other identified items) must be removed (Refer to [MH Search of a Patient, Property and Environment and Seizure of Property Procedure](#)).

Where a patient is in seclusion, it is essential to ensure that the patient is made aware of the method for calling for attention if needed and appropriate provision is made for their basic needs, including bedding, clothing, food, drink, and toilet facilities.

Patients should retain their own clothing unless there is a serious clinical contra- indication which may compromise safety. Such reasons must be documented in the patient's clinical record. Patients must not be secluded naked or in underclothes. Non-tear clothing may only be considered if there are extreme safety concerns. Refer to [MH Clinical Observation Procedure](#). In the event the use of non-tear garments is required without patient agreement, the treating or Duty Psychiatrist must approve this in consultation with the CNS, CNM or delegate.

Seclusion must be terminated as soon as it is safe to do so for both patient and other persons. For safety reasons, workers shall never enter a seclusion room alone or when it is deemed



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unsafe to do so. Assemble the required number of workers to break seclusion; for safety generally a minimum of three workers are required (Refer to PART training).

While in seclusion the patient must be observed by an mental health practitioner, or registered or enrolled nurse every 15 minutes, including monitoring of respiration rate. The observations will be documented on the **Form 11D** relevant to the reason for seclusion being required to continue. The reason for the regular observations is also to note any change in the patient's condition and alert workers if the patient's health suddenly deteriorates while in seclusion. Every 15-minute observation is an opportunity to assess the need for seclusion and to cease.

The 15-minute observations are undertaken from outside the seclusion room and preferably direct observation via a viewing panel in the door or an adjacent viewing area (depending on design of seclusion area) rather than through a closed-circuit television (CCTV) link, as a CCTV link may provide an inadequate impression of the condition of the patient.

In addition to the 15-minute observations, CCTV (closed circuit television), if available, should be activated once seclusion has commenced to allow the shift coordinator or ward staff to monitor progress and provide support for the nurse undertaking the clinical observations of the patient in seclusion.

The nurse in charge of the ward is to ensure that only workers that are required to view the CCTV monitor will access it throughout seclusion to protect the dignity of the patient. The patient will be informed at the commencement of seclusion that CCTV facilities are used in the seclusion process.

If CCTV monitoring is not available, regular physical contact must be maintained with the nurse undertaking the clinical observations.

Whilst observing a patient in seclusion, the identified mental health practitioner, registered or enrolled nurse must be aware that this duty is a priority. In the event of an emergency, the allocated person must ensure that the patient remains under observation in seclusion and summon assistance if evacuation or other action is required.

Wherever possible, a clock should be visible for the patient to refer to and assist with their orientation. The patient shall be aware of the means of calling for attention or assistance.

A medical practitioner must examine the person at least every 2 hours and more frequently when the condition of the patient indicates medical intervention is appropriate.

The purpose of the medical practitioner examination is to ensure that the patient's medical condition is satisfactory and to assess whether seclusion should continue or be revoked. Details of these examinations should be put on the extending Seclusion Form (**Form 11E**).

At any time, a medical practitioner, a mental health practitioner or the person in charge of the ward can revoke the seclusion order and the patient must be allowed to exit the seclusion area (**Form 11F**).

Seclusion must be revoked as soon as reasonably practicable after the patient has regained control of their behaviours and is able to follow staff direction.

A risk assessment must be conducted prior to a decision to revoke or the expiry of the seclusion order. This can be in the form of clinical discussion and should be documented in the patient's integrated notes.



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## 8.8 Management of Patient Post a Restraint and/or Seclusion Incident

Following the release of a patient from restraint and/or seclusion, a medical practitioner will carry out a physical examination within 6 hours to ensure that there is no complication of, or deterioration in, the patient's mental or physical condition that is or may be a result of the patient being restrained (**Form 10I**) or secluded (**Form 11G**).

Following restraint and/or seclusion, all patients must be offered support and counselling preferably from a person of their choice. Support may also include access to the Mental Health Advocacy Service.

Aboriginal MH staff (where appropriate) should ensure the cultural welfare and safety of the patient following seclusion wherever practicable/appropriate and/or the Peer Support Worker should also be considered.

A Post Restraint/ Seclusion Interview must be offered to the patient and is to be conducted as soon as practicable post the incident by a worker chosen by the patient. If it is in accordance with the patient's wishes and care plan, the Peer Support Worker, [Aboriginal Mental Health Liaison Officer](#), family/support person/ carer or nominated person should be included in the Post Restraint/Seclusion Interview.

The information gathered by the Post Restraint/Seclusion Interview is to be recorded in the Post Restraint/Seclusion Interview form (**PMR 43A**) and recorded in the patient's clinical record.

The CNS/Nurse in Charge of the Ward (after hours the most senior registered nurse on duty) is responsible for ensuring the patient's risk assessment, TSDP and Coping and Safety Tool for Awareness and Recovery (**PMR 10**) is reviewed and updated as part of the clinical review process in collaboration with the patient and/or their carer or nominated person as soon as practicable post restraint and/or seclusion.

Patients have a right to refuse to engage in a Post Restraint/Seclusion Interview, but every attempt should be made by workers to engage the patient in the Post Restraint/Seclusion Interview process. If a patient chooses not to engage in a Post Restraint/Seclusion Interview this should be documented on the Post Restraint/Seclusion Interview form (**PMR 43A**) and the patient's integrated notes and reattempted if appropriate/possible..

## 9. Documentation

The relevant MHA 2014 forms (**Form 10A, Form 10B, Form 10C, Form 10 D, Form 10E** (where applicable), **10F, 10 G, GH** (where applicable) and **10I**) must be completed for **every** occasion of bodily restraint, regardless of if a seclusion follows. If seclusion occurs then in addition, the MHA 2014 forms pertaining to the seclusion are required to be completed (Forms **11A, 11B, 11c, 11D, 11E, 11F, 11G**).

Datix CIMS online notification form must be completed following every incident of restraint and/or seclusion. The incident must be rated as a SAC 2. If there is a serious adverse outcome a SAC 1 rating will be required.

All forms must be checked by the CNS to ensure they are comprehensively completed and comply with the *MHA 2014* legislative requirements prior to submitting to NMHS MH Safety Quality & Performance Unit and, where applicable, Mentally Impaired Accused Review Board.



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Local services are to maintain their own seclusion and restraint register and follow the MH Processing of Seclusion and Restraint Forms (**Appendix 1**).

A copy of all the forms used in the restraint and/or seclusion process must be given to the patient unless it is determined by the medical officer that to do so may not be in the best interest of the patient or others. If the forms are not provided to the patient, the medical officer will document the reasons for this decision in the patient's clinical record. The forms for the patient will be provided by the Clinical Nurse Specialist or delegate who has ensured they are de-identified (worker names).

A copy of all restraint forms to be forwarded to the Nurse Manager or equivalent once all of the forms are checked and completed in full. Restraint forms must not leave the clinical area until all forms are completed accurately and correctly.

The patient's TSDP Plan and the Coping and Safety Tool for Awareness and Recovery PMR 10 will be reviewed and updated after every incident of restraint/seclusion.

PSOLIS Alerts and Incidents will be reviewed and updated (Refer to [MH Management of Clinical Alerts Policy](#)).

PMR 001 Clinical Alert Form will be reviewed and updated.

The original Post Seclusion/ Restraint Interview Form (PMR 43A) is to be placed in the patient's clinical record. Copies of all forms will be presented at the executive review of the incident, including the Post Seclusion/ Restraint Interview Form (PMR 43A)

A copy of **all** seclusion forms (**Forms 11A, 11B, 11C, 11D, 11E, 11F, 11G**) will be provided to the Chief Psychiatrist after any seclusion order. Forms will be sent to the OCP via My File Transfer (MyFT) as per the [MH Patient Information – Electronic Transmission of Guideline](#)

The relevant Clinical Nurse Specialist (CNS) (after hours, the most senior registered nurse on duty) will document in the patient's medical record:

- Guidance on when to cease seclusion.
- What action is required post-seclusion, including completion of necessary documentation
- Post-seclusion alterations to the Management Plan e.g. review of observation level.

The Medical Practitioner will record in the patient's clinical record date, time and results of each face to face medical examination of the patient in the 24 hours following seclusion.

The Mental Health Practitioner or nurse in charge of the ward will ensure that the patient's medical record reflects:

- Identifiable factors which may have contributed to seclusion
- Actions taken by workers prior to seclusion to defuse situation to avoid seclusion
- Length of seclusion and any breaks in seclusion
- Patient's post-seclusion presentation e.g. asleep, settled, cooperative, etc
- Documentation completed or yet to be completed, including Post Seclusion Interview (**PMR 43A Appendix 2**).

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### Form 12C Restriction on Freedom of Communication:

- Document reasons why the patient's communications have been limited or restricted in the integrated progress notes in the patient's medical record.
- The Shift Coordinator will indicate in the ward report book or equivalent, that the patient's right to freedom of lawful communication has been limited or restricted.
- When restrictions have been lifted, the original Form 12C will be forwarded to the Mental Health Tribunal via the Release of Information Officer and a copy placed in the patient's medical record.

## 10. Transfer of Patients

If a patient is required to be moved from one area to another as part of the process for managing a potentially violent or aggressive situation, an appropriate vehicle will be used where available/required, in accordance with the Department of Health [Requesting Road-Based Transport for Mental Health Patients Subject to Transport Orders Policy](#). The patient's head and or neck must not be held during this process. If possible, the clinical staff member/s accompanying the patient should be selected on the basis of their rapport with the patient and their skills and competencies in the management of violence and/or aggression.

Refer to WA Department of Health [Clinical Handover Policy](#) and the WA Department of Health [Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Patients Policy](#).

In accordance with the patient's consent and care plan, a carer, close family member or nominated person should be informed 'as soon as practical' of the transfer of the patient to another unit/hospital. This contact is to be documented in the patient's clinical record, identifying the carer and the time of the contact. If the patient does not wish to have the responsible person informed this is to be documented in the patient's medical record.

## 11. Executive Review of Restraint and/or Seclusion and Restrictive Practices

An Executive Review of Restraint is convened weekly and includes the Head of Clinical Service (HOCS), Coordinator of Nursing, Nurse Manager or Clinical Nurse Specialist, Service Manager, Treating Consultant and Peer Support Worker, within seven (7) days of the event occurring or as soon as practical. Complex incidents or cases are also escalated for executive review along with other restrictive practices.

This includes:

- Escalation through the relevant Co-Director to the Clinical Incident Review Committee (CIRC) for:
  - patients who have multiple seclusions or restraints within one week;
  - restraint or seclusion for > 6 hours;
  - an event which results in serious injury to the patient or workers.

For complex cases, where alternative strategies identified in the case conference are not effective, the Co-Director will escalate to the Director of Medical Services and Director of

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Nursing for an independent review. The Medical Co Director will ensure evidence of local interventions and reviews are available during the independent review.

Outcomes of each Executive review will be recorded on the relevant MH Seclusion and Restraint database, DATIX and in the patient's medical record.

In addition, each service will utilise this forum to examine any practices escalated for their consideration as being a Restrictive Practice (usually environmental).

The review is not a blaming process and should include workers involved in the incident wherever possible. Restraint and seclusion data will be displayed on Quality Boards for both staff and patients where possible.

## 12. Debriefing and Reporting Incidents

Workers are provided with post incident debriefing, with Coordinating Clinicians responsible to ensure this has occurred. Information and resources are available to access from [NMHS Health Point Critical Incident Response](#).

A formal [Critical Incident Team Debrief](#) can be organised by the Nurse Manager/Clinical Nurse Specialist for workers within 72 hours as required. All workers should be included in offers of debriefs, including Security, Allied Health and Medical workers.

[Employee Assistance Program](#) (EAP) contact information is available on the NMHS Health Point EAP page and posters and pamphlets are available in staff areas for workers to request individual debriefing and support as required.

Incidents in which workers have been harmed or may have been harmed, either physically and/or psychologically, should be reported to the [Health, Safety and Wellbeing \(HSW\) Department](#). These can be reported via [Incident Reports](#) or by directly contacting the [HSW Department](#) for immediate assistance. These reports enable response to both single incidents and also identification of high risk areas to increase support activities.

## 13. Compliance and Evaluation

100% of restraint and/or seclusion incidents will be recorded on Datix-CIMS and investigated in accordance with service requirements for the management of Datix-CIMS.

Leadership Committees will receive clinical data reports and Mandatory Training Compliance Data for their services.

MHPHDS Safety, Quality and Performance Unit is responsible for providing audit reports to the MHPHDS Executive (Link to [SQP Audit tools](#)).

MHPHDS Safety, Quality Audit and Performance Officer is responsible for providing a copy of all restraint and/or seclusion forms to the Chief Psychiatrist after any incident.

MHPHDS Safety, Quality Audit and Performance Officer is responsible for providing 3 monthly restraint and/or seclusion reconciliation reports to the Office of the Chief Psychiatrist.

All Form 12c's are to be reviewed every 24 hours by a Consultant Psychiatrist.

Leadership Committees will receive monthly clinical data reports for their services and monitor compliance with Executive Reviews of Restraint and/or seclusion in services. Leadership Committees will review incident reports and worker injuries on a regular basis.



## Related internal policies, procedures and guidelines

[NMHS Use of Force Policy](#)

[MH Clinical Risk Assessment and Management Policy](#)

[MH Search of a Patient, Property and Environment and Seizure of Property Procedure](#)

[MH Clinical Observation Procedure.](#)

[MH Management of Clinical Alerts Policy](#)

[MH Patient Information – Electronic Transmission of Guideline](#)

[NMHS Health Point Critical Incident Response](#)

## References

[MHA 2014](#)

[Chief Psychiatrist – Clinicians’ Practice Guide to the Mental Health Act 2014](#)

[WA Health Code of Conduct Policy](#)

[Chief Psychiatrist’s Standards for Clinical Care](#)

[Work Health and Safety Act 2020](#)

[Work Health and Safety Regulations 2022](#)

[Chief Psychiatrist’s Guideline for the Sexual Safety](#)

[WA DoH State-wide Standardised Clinical Documentation](#)

[WA Chief Psychiatrist’s Standards for Clinical Care](#)

[\(NSQHS\) Standards and User guide for acute and community mental health services](#)

[WA DoH Use of Physical and/or Mechanical Restraint during Road-based Transportation of Mental Health Patients Policy](#)

## Useful resources

[MHA Forms](#)

[Aboriginal Mental Health Liaison Officer](#)

[MH Critical Incident Team Debrief](#)

[Employee Assistance Program](#)

[NMHS Health, Safety and Wellbeing \(HSW\) Department](#)

[NMHS Incident Reports](#)

[SQP Audit tools](#)

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|--------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|------------|--------------|------------|
| Policy Sponsor                                                                             | Director of Medical Services MHPHDS                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |                |            |              |            |
| Policy Contact                                                                             | Co-Director Adult Inpatient Services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |                |            |              |            |
| Date First Issued:                                                                         | 28/06/2024                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | Last Reviewed: | 28/06/2024 | Review Date: | 28/06/2027 |
| Approved by:                                                                               | MH Policy Coordination Committee                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |                |            | Date:        | 21/06/2024 |
| Endorsed by:                                                                               | MHPHDS Executive Director                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |                |            | Date:        | 28/06/2024 |
| NSQHS Standards Applicable:                                                                | <div><div><div><input checked="" type="checkbox"/>  Std 1: Clinical Governance</div><div><input checked="" type="checkbox"/>  Std 2: Partnering with Consumers</div><div><input type="checkbox"/>  Std 3: Preventing and Controlling Healthcare Associated Infection</div><div><input type="checkbox"/>  Std 4: Medication Safety</div></div><div><div><input type="checkbox"/>  Std 5: Comprehensive Care</div><div><input checked="" type="checkbox"/>  Std 6: Communicating for Safety</div><div><input type="checkbox"/>  Std 7: Blood Management</div><div><input type="checkbox"/>  Std 8: Recognising and Responding to Acute Deterioration</div></div></div> |                |            |              |            |
| Chief Psychiatrist's Standards for Clinical Care:                                          | Physical Health Care of Mental Health Consumers<br>Risk Assessment and Management.<br>Seclusion and Bodily Restraint Reduction                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                |            |              |            |
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| Document Version Control |         |                                 |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|--------------------------|---------|---------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Date                     | Version | Author                          | Notes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| 28/06/2024               | V1.0    | Director of Medical Services MH | New policy and procedure to provide direction for workers in the use of physical restraint and seclusion in authorised MH Services as well as providing a framework for ensuring other restrictive practices do not occur within Inpatient areas. Early assessment and intervention, collaboration, crisis management, de-escalation and other supportive alternatives are preferred interventions of choice in delivering safe, high quality patient care whilst also maintaining a safe workplace. This new policy and procedure incorporates the MH Seclusion and MH Restraint Policies. |
|                          |         |                                 |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|                          |         |                                 |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD 3443).

This document can be made available in alternative formats on request for a person with a disability.

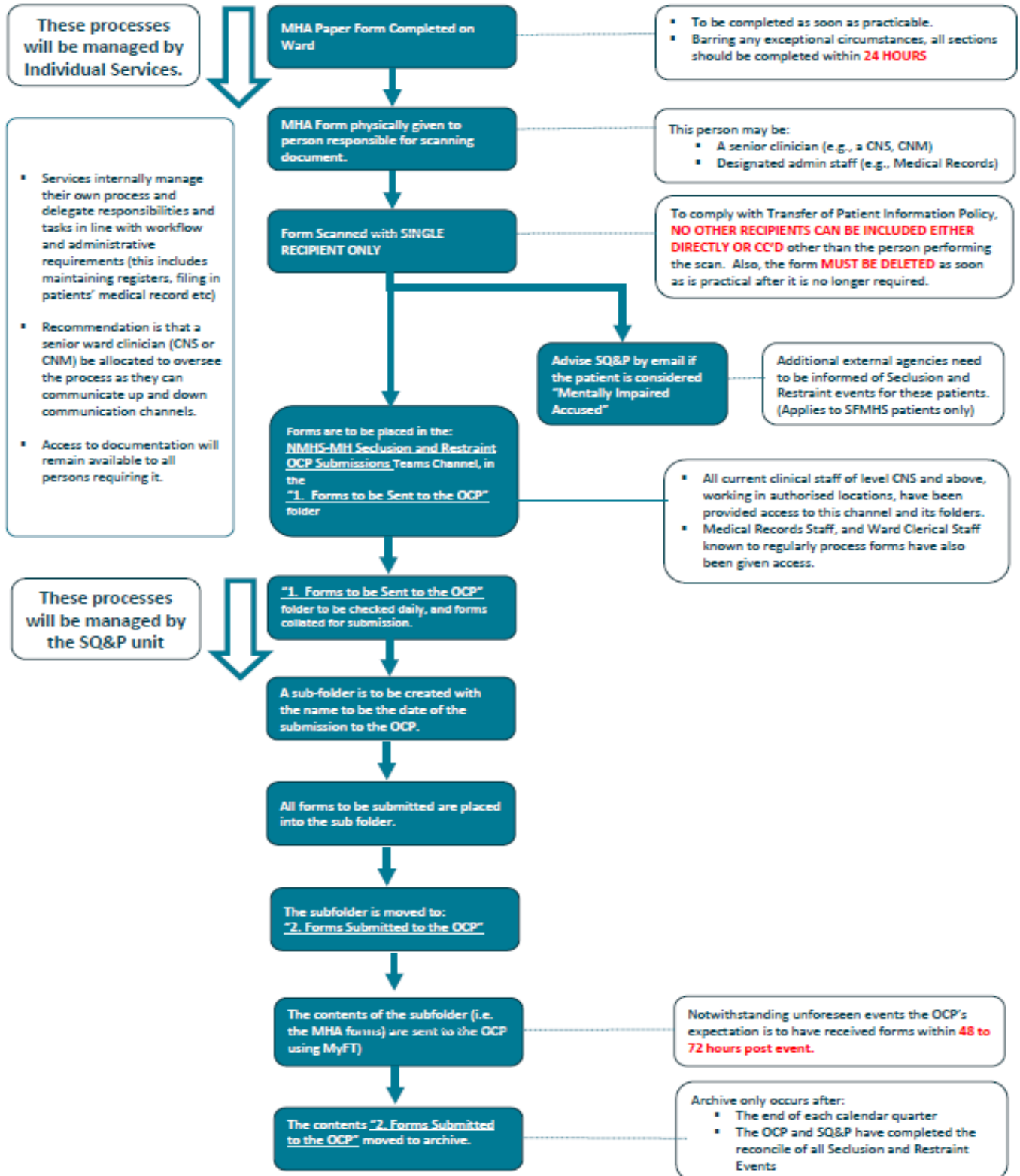
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## 14. Appendix 1: MH Seclusion and Restraint Forms Process – Flowchart

### MHPHDS-MH Processing of Seclusion and Restraint Forms:



## Appendix 2: PMR 43A Post Seclusion Patient Interview

|                                                                                                                                                      |                                       |           |            |         |
|------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------|-----------|------------|---------|
| <b>NORTH METROPOLITAN AREA HEALTH SERVICE</b><br><b>MENTAL HEALTH</b><br><br>..... Hospital<br><br><b>POST SECLUSION</b><br><b>PATIENT INTERVIEW</b> | Surname                               |           | Sex        | U.R. No |
|                                                                                                                                                      | Forenames                             |           | D.O.B.     |         |
|                                                                                                                                                      | Address                               |           |            |         |
|                                                                                                                                                      | Ward                                  | Registrar | Consultant |         |
|                                                                                                                                                      | Use Patient I.D. Label when available |           |            |         |

**Date/Time seclusion ended** .....

In the last 48 hours a decision was made by staff to seclude you. We regret that this occurred. Your suggestions and thoughts are important to help us better manage similar situations safely without the use of seclusion or restraint. A staff member will assist you to complete this form. Extra pages can be attached if necessary.

The Treating Team will work with you to help you feel safe and ensure the safety of others whilst you are a patient here. Seclusion and restraint may be used to protect yourself or others from imminent and/or immediate risk of harm. Please remember to include anything you may find helpful if such a situation arises.

☐ ..... Please tick box and insert date if patient refuses to partake in process.

1. Can you tell me what you think may have led to you being secluded/restrained?  
 .....  
 .....

2. Prior to the seclusion/restraint how are you feeling?  
 Please describe .....  
 .....  
 .....

3. Is there something the staff could try in the future to assist you if you feel this way again? Yes No  
 If yes please list .....  
 .....  
 .....

4. What alternatives to seclusion/restraint were tried? Did any of them help you?  
 .....  
 .....

5. Was your privacy and dignity respected during the incident? Yes No  
 If no please describe .....  
 .....  
 .....

6 Is there anything we can do to assist you recover from this incident? Yes No  
 If yes please describe .....  
 .....  
 .....

Patient Name & Signature ..... Date: .....

Staff Name & Signature ..... Date: .....

PMR 43A POST SECLUSION PATIENT INTERVIEW

## PMR 43A Post Seclusion Patient Interview (Page 2)

|                                                                                                                                                                                |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------|-----|---------|-----------|--|--------|---------|--|--|------|-----------|------------|---------------------------------------|--|--|
| <b>NORTH METROPOLITAN AREA HEALTH SERVICE<br/>MENTAL HEALTH</b><br><br>.....Hospital<br><br><div style="text-align: center;"><b>POST SECLUSION<br/>PATIENT INTERVIEW</b></div> | <table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 50%;">Surname</td> <td style="width: 10%;">Sex</td> <td style="width: 40%;">U.R. No</td> </tr> <tr> <td colspan="2">Forenames</td> <td>D.O.B.</td> </tr> <tr> <td colspan="3">Address</td> </tr> <tr> <td>Ward</td> <td>Registrar</td> <td>Consultant</td> </tr> <tr> <td colspan="3">Use Patient I.D. Label when available</td> </tr> </table> | Surname    | Sex | U.R. No | Forenames |  | D.O.B. | Address |  |  | Ward | Registrar | Consultant | Use Patient I.D. Label when available |  |  |
| Surname                                                                                                                                                                        | Sex                                                                                                                                                                                                                                                                                                                                                                                                                                       | U.R. No    |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Forenames                                                                                                                                                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                           | D.O.B.     |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Address                                                                                                                                                                        |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Ward                                                                                                                                                                           | Registrar                                                                                                                                                                                                                                                                                                                                                                                                                                 | Consultant |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Use Patient I.D. Label when available                                                                                                                                          |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |

**This section to be completed by staff**

**Aim**

- To assure the patient that we regret the restraint/seclusion occurred
- To advise the patient we are trying to reduce the use of restraint/seclusion within our service
- To engage the patient in identifying ways to reduce and where possible avoid the use of restraint/seclusion

**Action**

The patient's treating team should decide the best person to conduct the post seclusion interview and address the following points with the patient:

- Customise approach, i.e. setting, attention span, memory etc
- Avoid any blame, shame or lectures
- Feedback following the interview to be given to all staff by CNS
- Management plan updated and outcomes documented in all patients' Integrated Progress Notes

**Planning**

- What new information do we have and how can we use it to avoid a similar situation in the future?

**Issues identified from seclusion and restraint interview**

- Patient Factors**  
.....  
.....  
.....
- Policy and Procedure Factors**  
.....  
.....  
.....
- Environmental Factors**  
.....  
.....  
.....
- Staff training/skill factors**  
.....  
.....  
.....

**Action to be undertaken for the patient**

|                                                                              |                                                                                                                                                              |                                                                                                          |
|------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------|
| Patients management plan updated<br><br><br>By whom Name .....<br>Date ..... | Request to see member of multidisciplinary team actioned i.e. name of person advised & by whom<br>Person contacted .....<br>By whom Name .....<br>Date ..... | Treating psychiatrist/team updated<br><br><br>Person contacted .....<br>By whom Name .....<br>Date ..... |
|------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------|

## Appendix 3: Coping and Safety Tool for Awareness and Recovery (Patient Safety Plan PMR 10)

|                                                                                                                                                                                                    |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------|-----|---------|-----------|--|--------|---------|--|--|------|-----------|------------|---------------------------------------|--|--|
| <b>NORTH METROPOLITAN HEALTH SERVICE</b><br><b>MENTAL HEALTH</b><br>..... Hospital<br><b>COPING AND SAFETY TOOL</b><br><b>FOR AWARENESS</b><br><b>AND RECOVERY</b><br><b>(Patient Safety Plan)</b> | <table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 40%;">Surname</td> <td style="width: 10%;">Sex</td> <td style="width: 50%;">U.R. No</td> </tr> <tr> <td colspan="2">Forenames</td> <td>D.O.B.</td> </tr> <tr> <td colspan="3">Address</td> </tr> <tr> <td>Ward</td> <td>Registrar</td> <td>Consultant</td> </tr> <tr> <td colspan="3">Use Patient I.D. Label when available</td> </tr> </table> | Surname    | Sex | U.R. No | Forenames |  | D.O.B. | Address |  |  | Ward | Registrar | Consultant | Use Patient I.D. Label when available |  |  |
| Surname                                                                                                                                                                                            | Sex                                                                                                                                                                                                                                                                                                                                                                                                                                       | U.R. No    |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Forenames                                                                                                                                                                                          |                                                                                                                                                                                                                                                                                                                                                                                                                                           | D.O.B.     |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Address                                                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Ward                                                                                                                                                                                               | Registrar                                                                                                                                                                                                                                                                                                                                                                                                                                 | Consultant |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Use Patient I.D. Label when available                                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |

The Coping and Safety Tool is intended to be completed by you with your allocated nurse present who can help you to complete the plan and answer any questions. This Coping and Safety Tool will suggest things you can do, as well as what staff can do, to help you feel less stressed or anxious whilst you are receiving care and treatment. It will help us all to focus on preventing a crisis and promoting a safe environment for you and others. The tool allows you to identify the things (called 'triggers') and early warning signs for when you are becoming angry or upset. It also helps you to list ways of coping that are helpful to you during these times. We can use this tool to assist you with your treatment, care and recovery. You can add or take away things from your Coping and Safety Tool at anytime.

**How to use this tool: Please write, tick or circle your responses as indicated.**

What does coping and safety mean to you? \_\_\_\_\_

\_\_\_\_\_

\_\_\_\_\_

**History**  
 In the past, have you experienced any of the following? (You do not need to disclose or give details of past events).

|                                    |  |                      |  |
|------------------------------------|--|----------------------|--|
| Feeling unsafe                     |  | Aggressive behaviour |  |
| History of trauma/traumatic events |  | Depression/sadness   |  |
| Feeling suicidal/suicide attempts  |  | Anxiousness          |  |
| Being in prison                    |  | Other                |  |
| Restraint or seclusion             |  | Other                |  |
| Self-harming behaviours            |  | Other                |  |

Would you like to say more on any of these issues? Are these particularly relevant when you are in hospital? \_\_\_\_\_

\_\_\_\_\_

**Identify Triggers**  
 In the past, what do you think has made you feel angry or upset?

|                                      |  |                                             |  |
|--------------------------------------|--|---------------------------------------------|--|
| Not being listened to                |  | Being isolated                              |  |
| Feeling lonely                       |  | Being touched                               |  |
| Name calling or being made fun of    |  | Particular time of day or night             |  |
| Being threatened or feeling unsafe   |  | Hearing voices or experiencing bad thoughts |  |
| People yelling/people yelling at you |  | Hanging out (for substances)                |  |
| Arguments                            |  | Feeling depressed                           |  |
| Missing out on important events      |  | Feeling intimidated                         |  |
| Feeling bored                        |  | Being around certain people                 |  |
| Bedroom door open                    |  | Loud noises                                 |  |
| Personal space invaded               |  | Not having control                          |  |
| Particular smells                    |  | People in uniform / ward staff              |  |
| Being tired                          |  | Being told what to do                       |  |
| Being physically unwell              |  | Contact with family or friends              |  |
| Feeling disrespected                 |  | Anniversary/significant event               |  |
| Not being able to smoke              |  | Other                                       |  |
| Being hungry/thirsty                 |  | Other                                       |  |

PMR 10 COPING AND SAFETY TOOL FOR AWARENESS AND RECOVERY

## Coping and Safety Tool for Awareness and Recovery (Patient Safety Plan PMR 10) – Page 2

|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |           |                                     |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
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| <b>NORTH METROPOLITAN HEALTH SERVICE<br/>MENTAL HEALTH</b><br><br><b>COPING AND SAFETY TOOL<br/>FOR AWARENESS<br/>AND RECOVERY<br/>(Patient Safety Plan)</b>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |           |                                     |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
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|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | Address   |                                     |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
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| Use Patient I.D. Label when available                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |           |                                     |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| <p>Would you like to say more about any of these triggers? _____</p> <p>_____</p> <p>_____</p> <p>Are there any other issues we should know about? (e.g. pets at home, worries about children, bills due, work, study etc?) _____</p> <p>_____</p> <p>_____</p> <p><b>Early Warning Signs</b></p> <p>Do you find any of the following warning signs relevant to you? This may help you and others recognise when you begin to feel out of control.</p> <table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 40%;">Changes in sleep (more/less)</td> <td style="width: 10%;"></td> <td style="width: 40%;">Changes in thinking/racing thoughts</td> <td style="width: 10%;"></td> </tr> <tr> <td>Clenching fists/teeth</td> <td></td> <td>Worrying a lot/thinking too much</td> <td></td> </tr> <tr> <td>Yelling/swearing</td> <td></td> <td>Unable to sit still</td> <td></td> </tr> <tr> <td>Being rude/abusive</td> <td></td> <td>Heart racing/dry mouth</td> <td></td> </tr> <tr> <td>Hurting myself</td> <td></td> <td>Withdrawing/isolating myself</td> <td></td> </tr> <tr> <td>Pacing</td> <td></td> <td>Not taking care of myself</td> <td></td> </tr> <tr> <td>Wanting to hit or throw objects</td> <td></td> <td>Changes in eating (more/less)</td> <td></td> </tr> <tr> <td>Slamming doors</td> <td></td> <td>Avoiding people</td> <td></td> </tr> <tr> <td>Feeling irritable or angry</td> <td></td> <td>Other</td> <td></td> </tr> <tr> <td>Hearing voices/visual changes</td> <td></td> <td>Other</td> <td></td> </tr> <tr> <td>Talking to myself/the voices</td> <td></td> <td>Other</td> <td></td> </tr> </table> <p>Would you like to say more on any of these early warning signs? _____</p> <p>_____</p> <p>_____</p> <p>How can you let staff know that you may be struggling? _____</p> <p>_____</p> <p>_____</p> <p>Preference for male/female staff – particularly after a distressing experience? YES / NO (please circle)</p> <p><input type="checkbox"/> Male      <input type="checkbox"/> Female      <input type="checkbox"/> Mix of staff      <input type="checkbox"/> No preference</p> <p><i>We will do what we can to accommodate your specific needs.</i></p> <p>Would you like a family member/someone else contacted after a distressing experience? YES / NO (please circle)</p> <p>Name of contact person: _____</p> <p>Contact information: _____</p> <p>When can this person be contacted? Anytime or best time: _____</p> |           |                                     |            | Changes in sleep (more/less) |  | Changes in thinking/racing thoughts |  | Clenching fists/teeth |  | Worrying a lot/thinking too much |  | Yelling/swearing |  | Unable to sit still |  | Being rude/abusive |  | Heart racing/dry mouth |  | Hurting myself |  | Withdrawing/isolating myself |  | Pacing |  | Not taking care of myself |  | Wanting to hit or throw objects |  | Changes in eating (more/less) |  | Slamming doors |  | Avoiding people |  | Feeling irritable or angry |  | Other |  | Hearing voices/visual changes |  | Other |  | Talking to myself/the voices |  | Other |  |
| Changes in sleep (more/less)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |           | Changes in thinking/racing thoughts |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Clenching fists/teeth                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |           | Worrying a lot/thinking too much    |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Yelling/swearing                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |           | Unable to sit still                 |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Being rude/abusive                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |           | Heart racing/dry mouth              |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Hurting myself                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |           | Withdrawing/isolating myself        |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Pacing                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |           | Not taking care of myself           |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Wanting to hit or throw objects                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |           | Changes in eating (more/less)       |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Slamming doors                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |           | Avoiding people                     |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Feeling irritable or angry                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |           | Other                               |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Hearing voices/visual changes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |           | Other                               |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |
| Talking to myself/the voices                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |           | Other                               |            |                              |  |                                     |  |                       |  |                                  |  |                  |  |                     |  |                    |  |                        |  |                |  |                              |  |        |  |                           |  |                                 |  |                               |  |                |  |                 |  |                            |  |       |  |                               |  |       |  |                              |  |       |  |



## Coping and Safety Tool for Awareness and Recovery (Patient Safety Plan PMR 10) – Page 3

|                                                                                                                                                                                                  |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
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| <p><b>NORTH METROPOLITAN HEALTH SERVICE</b><br/><b>MENTAL HEALTH</b></p> <p>..... Hospital</p> <p><b>COPING AND SAFETY TOOL<br/>FOR AWARENESS<br/>AND RECOVERY<br/>(Patient Safety Plan)</b></p> | <table border="1" style="width: 100%; border-collapse: collapse;"> <tr> <td style="width: 50%;">Surname</td> <td style="width: 10%;">Sex</td> <td style="width: 40%;">U.R. No</td> </tr> <tr> <td colspan="2">Forenames</td> <td>D.O.B.</td> </tr> <tr> <td colspan="3">Address</td> </tr> <tr> <td>Ward</td> <td>Registrar</td> <td>Consultant</td> </tr> <tr> <td colspan="3">Use Patient I.D. Label when available</td> </tr> </table> | Surname    | Sex | U.R. No | Forenames |  | D.O.B. | Address |  |  | Ward | Registrar | Consultant | Use Patient I.D. Label when available |  |  |
| Surname                                                                                                                                                                                          | Sex                                                                                                                                                                                                                                                                                                                                                                                                                                       | U.R. No    |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Forenames                                                                                                                                                                                        |                                                                                                                                                                                                                                                                                                                                                                                                                                           | D.O.B.     |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Address                                                                                                                                                                                          |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Ward                                                                                                                                                                                             | Registrar                                                                                                                                                                                                                                                                                                                                                                                                                                 | Consultant |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |
| Use Patient I.D. Label when available                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                                                                                                                                           |            |     |         |           |  |        |         |  |  |      |           |            |                                       |  |  |

**Coping strategies:**  
What is helpful (and safe) for you?  
What are some of the things that calm you down or keep you safe?

|                                          |                                              |  |
|------------------------------------------|----------------------------------------------|--|
| Talk with me                             | Going for a walk                             |  |
| Let me sit quietly in a room by myself   | Having a bath or shower                      |  |
| Breathe deeply/relaxation exercises      | Exercise                                     |  |
| Offer me another activity to do          | Artwork/craft activities                     |  |
| Offer medication                         | Having food/drink                            |  |
| Listening to music/dvd/Wii               | Talk with other consumers/Peer Support staff |  |
| Watching TV                              | Reading e.g. book, magazine, newspaper       |  |
| Calling friends or family                | Flicking through a magazine/paper            |  |
| Playing video games/computer             | Using stress balls                           |  |
| Playing a board game/cards               | Using scented hand lotion                    |  |
| Writing (e.g. letter, poem, diary)       | Shredding paper                              |  |
| Using the courtyard/being outside        | Wrapped in blanket/tucked in                 |  |
| Playing a musical instrument e.g. guitar | Playing with tennis/soccer ball/sport        |  |
| Using rubberband on wrist                | Using NRT options                            |  |
| Running cold/warm water on hands         | Having a cigarette                           |  |
| Singing                                  | Listening to a relaxation CD                 |  |
| Using/sitting on exercise ball           | Guided relaxation                            |  |
| Chewing gum                              | Incense or nice smells                       |  |
| Contact with family/friends              | Sitting in a darkened room                   |  |
| Eating sour lollies                      | Other                                        |  |

Would you like to say more about any of these coping strategies? \_\_\_\_\_

\_\_\_\_\_

**TOP THREE COPING STRATEGIES**

1. \_\_\_\_\_ 2. \_\_\_\_\_ 3. \_\_\_\_\_

Having completed this tool, how do you feel about coping and safety now?  
Is there anything else we can put in place so that you will feel safe and get the most from your stay in hospital?

\_\_\_\_\_

\_\_\_\_\_

Have you seen a Social Worker (SW): ☐ Yes ☐ No (Nurse to refer to SW)

\_\_\_\_\_

Consumer Signature \_\_\_\_\_ Staff Name and Signature \_\_\_\_\_ Date \_\_\_\_\_

☐ Original filed in medical record and Copy to consumer